



February 23, 2026

Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP) and Office of the National Coordinator for Health Information Technology (ONC) (collectively, ASTP/ONC), Department of Health and Human Services.

Subject: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

*Submitted Electronically*

To Whom It May Concern:

The American Psychological Association Services, Inc. (APA) respectfully submits the following response to your Request for Information. APA Services is the companion organization of the American Psychological Association, which is the nation's largest scientific and professional nonprofit organization representing the discipline and profession of psychology, as well as over 190,000 members and affiliates who are clinicians, researchers, educators, consultants, and students in psychological science. APASI emphasizes that while AI offers transformative potential for diagnostic precision and administrative efficiency, its deployment must be governed by the foundational principles of psychological science. To ensure these tools are effective, ethical, and equitable, the federal government must prioritize a human-centric regulatory framework that safeguards patient safety and restores public trust.

APASI recognizes AI's immense potential to revolutionize critical sectors, especially health care. AI can enhance diagnostic precision, expand access to behavioral health treatment, and alleviate administrative burdens that cause provider burnout. However, a sizeable barrier to the development, deployment, and adoption of beneficial AI is the profound and justified lack of public trust. As the RFI notes, "foster[ing] public trust" is "critical" for adoption. This trust is fragile because the current market is flooded with unregulated, direct-to-consumer AI products that make deceptive claims and pose significant risks.

The RFI correctly identifies "lack of regulatory clarity" and "regulatory mismatches" as primary barriers. This ambiguity has created a digital Wild West that *hinders* the adoption of safe and effective AI tools. To *promote* innovation, we must address these gaps by establishing clear, evidence-based guardrails. This requires embedding psychological science—the study of human cognition, behavior, and emotion—into the entire AI lifecycle -- the end-to-end process of developing and deploying an AI system.

Below, we identify specific barriers and recommendations corresponding to the RFI's questions.

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## I. Regulation

HHS must establish a regulatory framework that prioritizes patient safety and public trust.

- **Prohibit Misrepresentation of AI as Licensed Professionals:** A primary barrier to trust is the "regulatory mismatch" allowing AI chatbots to misrepresent themselves as "psychologists" or "therapists". For instance, a chatbot on a popular platform presenting itself as a "psychologist," engaged in millions of chats and, in some cases, appeared to validate users' violent thoughts. Congress and HHS should make it illegal for AI to generate fraudulent credentials or misrepresent itself as a licensed professional.
- **Mandate Transparency and "Human-in-the-Loop":** AI should be regulated as a tool to augment, not replace, the clinical judgment and therapeutic relationships that are the bedrock of quality care. Regulations must mandate persistent disclosure that a user is interacting with an AI system and ensure a qualified human remains in the loop for high-stakes clinical decisions.
- **Strict Transparency and Informed Consent:** It is imperative that more is known about the data and methods used to train AI systems in the healthcare setting. Developers must also be required to clearly, conspicuously, and persistently disclose to users that they are interacting with an AI system, not a human. In clinical settings, authorities must obtain informed consent, so patients understand how AI tools like predictive algorithms are being used and specifically, what personally identifiable information is being retained and accountability for information being released to third parties.
- **Professional Competence and Liability:** Policy should support continuing education for the workforce, as professionals utilizing AI must maintain competence in the technologies they employ. Over-reliance on technology without proper training, oversight, and disclosure can lead to harm.

## II. Reimbursement

Payment policies must incentivize evidence-based, high-value interventions rather than mere engagement metrics.

- **Incentivize Safety and Efficacy over Volume:** Reimbursement should be tied to robust clinical outcome data—such as remission rates and improvements in health-related quality of life—rather than corporate metrics like user engagement or number of downloads.
- **Support Interdisciplinary Care:** Payment models should incentivize the use of AI to expand the reach of clinicians within a human-led care team, ensuring technology does not displace the essential therapeutic relationships that are the bedrock of quality care. This is essential in an environment where states are moving to limit the ability of qualified providers to use evidence-based technologies to aid in the delivery of care.

## III. Research & Development

The pace of AI development is far outpacing independent scientific research.



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- **Embed Psychological Science in the AI Lifecycle:** Policy should mandate that AI developers collaborate with psychological scientists during the design phase to mitigate risks related to cognitive bias and emotional manipulation. This same integration should take place among federal regulatory, investigatory, and research teams tasked with developing AI policies.
- **Mitigate Bias and Disparities:** AI models often reflect biases from non-representative training data, such as an algorithm that unfairly assigned lower risk scores to Black patients because it used healthcare costs as a proxy for illness. HHS must require rigorous, independent pre-deployment testing and ongoing monitoring for harms across diverse populations to ensure AI reduces, rather than entrenches, health disparities.
- **Investment in Independent Research:** HHS should prioritize significant federal investment in independent, longitudinal research to understand the long-term impacts of human-AI interaction on mental health and social development. This research must be free from corporate conflicts of interest and facilitate data collection and transparency requirements for companies seeking to provide health-care related products reliant on AI technologies.

#### IV. Data Privacy and Vulnerable Populations

Current privacy laws are "structurally incompatible" with AI's capability to infer intimate details from vast datasets.

- **Ensure HIPAA Compliance and Data Privacy:** Current privacy frameworks are structurally incompatible with AI's ability to infer intimate mental health details from collected data. HHS must establish a right to "mental privacy," safeguarding data that can indicate a person's emotional state without explicit patient consent. Furthermore, many current AI chatbots are not HIPAA compliant, leaving patient privacy and security of their health data unprotected by federal law.
- **"Safe-by-Default" Standards for Youth:** Protections for children and adolescents must be the default setting, not an opt-in. This includes limiting manipulative design features that exploit the "neurodevelopmental mismatch" of adolescence, characterized by high social reward seeking and low impulse control.

#### Conclusion

APASI believes AI holds the potential to create a more accessible, effective, and equitable healthcare system. The path to realizing this potential is not through deregulation, but through the creation of a clear, evidence-based framework that builds public trust. The current lack of regulatory clarity is the primary key concern to AI adoption.

We must ensure AI is a tool to *augment*, not replace, the clinical judgment and therapeutic relationships that are the bedrock of quality care. The APASI and its member scientists stand ready to collaborate with OSTP to develop and enact policies that advance these principles.



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Thank you again for the opportunity to weigh in on these questions. To the extent that our psychologists can serve as a resource to your office, please feel free to reach out to Corbin Evans (Deputy Chief of Advocacy for Science & Technology) at [CEvans@apa.org](mailto:CEvans@apa.org).

Sincerely,

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